WORKSHEET A

Vision-to-Goals Canvas

Startup: **HappyHealth – Home Medical Care, Karachi**	Team: **THEO – Idea/Planning Stage (just in head)**

Work top to bottom. Name your riskiest assumptions before you write any goals.

1. Vision in one line (5–10 years)

Our vision
Patients across Karachi (17.15M – DHA/Clifton posh affluent) will receive qualified, affordable, doctor-led healthcare at their residence – no hospital queues. Building Pakistan's most trusted home-care brand for elderly & overseas families (32.91% diabetes, 30.43% HTN).

2. Our stage today (tick one) – CURRENTLY JUST AN IDEA IN HEAD / PLANNING

Problem–solution fit	Product–market fit	Early scale
■ **SELECTED – IDEA STAGE** Do they have the problem? YES – Karachi 96% mobile, 70% internet, 78-89% OOP. 0 users yet – just idea.	■ NOT YET Will they pay and stay? To be proven – Lajpal RN Rs95k-120k, oladoc Rs200-5000 exists.	■ NOT YET Can we grow profitably? Future after 10 paid pilots.

Evidence: IDEA STAGE ONLY – 0 users, 0 revenue, 0 pilots, 1 founder. Secondary: Karachi 17.15M, global $343B→$1006B 2035 CAGR 10.3%, 32.9% diabetes, 30.4% HTN, competitors Human Healthcare 2022 DHA/Clifton, AKU USD packages, Holistic 120 staff.

3. Riskiest assumptions — if this belief is wrong, the startup dies

Finish the sentence "Our startup only works if...". Think about customers, payment, usage and reach.

#	Assumption – RESEARCH BASED (ignore handwritten)	How sure are we? (1=guess, 5=proven)
1	Our startup only works if affluent families in DHA/Clifton/PECHS/Gulshan will pay in advance @ Rs4500/shift (Rs80k-120k/mo RN) and stay >30 days – despite 52.6% private, 89% OOP market.	**1 = Pure guess – RED P0** Idea stage, no paying clients. Market shows Lajpal packages exist, but unproven for us.
2	Our startup only works if PMDC/PNC-verified nurses & doctors (check license, call previous hospitals, background check, senior doctor supervises first visit) will reach <2h in Karachi traffic with zone hubs.	**1 = Pure guess – RED P0** No nurses hired yet. Supply via Rozee/OLX but punctuality unproven.
3	Our startup only works if overseas families (UAE/UK/US) will trust and pay in USD $199-349/mo via Family Health Dashboard + WhatsApp updates + quarterly teleconference (AKU model).	**1 = Pure guess – RED P0** AKU & Human Healthcare prove 4.9/5 demand, but our brand 0 trust.

Scoring key — how sure are we?

1 – RED	2 – ORANGE	3 – YELLOW	4 – LIGHT GREEN	5 – DARK GREEN
Pure guess ■ IDEA STAGE	People said they would	Some use, small/free	Real behaviour small	Proven at scale

4. Candidate six-month goals — outcomes true by month 6, not activities

#	Candidate goal (what will be true by month 6?) – OUTCOME	Tests assumption #
1	**30 problem interviews (20 local DHA/Clifton/PECHS +10 overseas) – 80% say would pay @ Rs4500/shift.** Outcome: Problem validated.	Tests #1 Pay – Client has problem
2	**15 PMDC-verified nurses +3 GPs recruited – license checked, hospital reference, police check, senior doctor supervises first visit.** Outcome: Supply ready.	Tests #2 Reach on time
3	**MVP live: WhatsApp Business + booking site + Family Health Dashboard v1 + free 15-min onboarding + doctor within 48h.** Outcome: Product usable.	Tests #1 + #3 Overseas trust
4	**5 pilot families DHA/Clifton paid 15 days advance @ Rs4500/shift (Rs67.5k) with vitals & EMR documentation.** Outcome: Money in bank.	Tests #1 Pay on time + #2 Reach
5	**SHCC provisional license filed + Chughtai free sampling MoU (HB Rs550) + pharmacy partnership.** Outcome: Legal + partners ready.	Tests #2 + Regulatory
6	**10 families renew 2nd month (70%) + NPS>50 + 0 incidents + equipment loss <2% + case navigator.** Outcome: Retention proven.	Tests #1 Stay + #3 Trust